Current Affairs 14-04-2026

Credit - Drishti Ias

Maternal Healthcare in India

★ ★ ★ ☆ ☆

For Prelims: World Health Organization , Maternal Mortality Ratio , SDG Target 3.1 , Health and Wellness Centers , Anemia

For Mains: Maternal Mortality and SDG 3.1 targets, Public health infrastructure and governance challenges, Role of social determinants in health outcomes

Source: IE

Why in News?

A study in The Lancet Obstetrics, Gynaecology & Women's Health highlights that although India has reduced **maternal mortality significantly since 1990** , the progress has slowed after 2015, raising concerns over gaps in **maternal healthcare and the need for a more comprehensive approach** beyond mere access.

Summary

- Despite a significant decline in **maternal mortality (MMR)** in India, progress has slowed after 2015, with **regional disparities and preventable causes** still posing major challenges.
- Strengthening **healthcare quality, infrastructure, midwifery, and addressing social determinants like anemia and early marriage** is essential to achieve **SDG target 3.1** .

What are the Key Highlights of the Study on Maternal Healthcare?

- **Pace of Progress and Persistent Burden:** Despite maternal mortality dropping to a **fifth of its 1990 levels,** progress has worryingly slowed since 2015.
  - **India still accounts for one in 10 global maternal deaths** , with preventable causes like **haemorrhages** **, infections, and blood pressure-related disorders** remaining the primary culprits.
- **SDG Challenge and Regional Disparities:** Meeting the **Sustainable Development Goal (SDG) target 3.1** **of reducing maternal mortality to 70 deaths per 1,00,000 live births by 2030** is

hindered by severe regional imbalances.

  - While states like Kerala, Maharashtra, and Tamil Nadu have achieved an MMR below 70, others, such as Assam (195), Madhya Pradesh (173), Uttar Pradesh (167), and Bihar (118), lag behind the national average, reflecting regional disparities in maternal healthcare.
- **The Pandemic's Impact:** **Covid-19** exposed the system's fragility, diverting frontline workers and delaying essential antenatal visits and institutional deliveries.
- **Strengthening Healthcare Infrastructure:** The study emphasises the need for a decisive shift towards **strengthening primary and secondary healthcare systems.**
  - It also highlights that expanding medical infrastructure alone will be insufficient unless the **social determinants of health are adequately addressed.**

Maternal Mortality

- **Maternal Mortality:** It is defined as the death of a woman while pregnant or within **42 days of termination of pregnancy, from any cause** related to or aggravated by pregnancy or its management, excluding accidental or incidental causes. It is a key health indicator measuring risks associated with childbirth.
  - Globally, in 2023, approximately 260,000 women died during or following pregnancy, equating to over 700 preventable maternal deaths every day (one death every two minutes).
- **Maternal Mortality Ratio (MMR):** It is the number of maternal deaths per 100,000 live births in a given time period.
  - It is a key indicator used to assess the quality of maternal healthcare and access to medical services during pregnancy and childbirth.
- **India’s MMR:** India has made progress, with the MMR falling from 384 in 2000 to **103 in 2020, and further down to 80 in 2023,** an **86% drop since 1990,** surpassing the global average decline of 48%.

What are the Challenges Hindering Quality Maternal Healthcare in India?

- **Human Resource Crisis in Rural India: According to the Rural Health Statistics (RHS) 2021-22 , there is an alarming nearly 80% shortfall of specialized doctors (Surgeons, Obstetricians & Gynaecologists, Physicians, and Paediatricians) at Community Health Centres (CHCs).**
- **Underutilized Midwifery Cadre:** Unlike Scandinavian countries or the UK, where **midwives lead low-risk deliveries** , India's healthcare system remains highly doctor-centric.
  - The **Nurse Practitioner in Midwifery (NPM) initiative** is still in its nascent stages, leaving existing nurses severely overburdened.
- **Infrastructure and Supply Chain Deficits: Many designated First Referral Units (FRUs) lack 24x7 operational readiness for Emergency Obstetric Care (EmOC).**
  - **Postpartum Haemorrhage (severe bleeding) is the leading cause of maternal death in India.** Yet, secondary care centers frequently lack functional, fully stocked blood
  - banks or even basic blood storage units, leading to **fatal delays as patients are shuttled to**

tertiary urban hospitals.

- Intermittent shortages of life-saving, low-cost drugs such as Magnesium Sulfate (for eclampsia/seizures) and Oxytocin (for bleeding) cripple frontline workers' ability to stabilize mothers.

- **Quality vs. Commercialization Divide:** Quality care means *appropriate* care. According to the National Family Health Survey (NFHS-5), the rate of Caesarean sections in private health facilities in **India stands at an alarming 47.4%,** far exceeding the WHO's recommended ideal rate of 10-15%.
  - This **over-medicalization exposes women to unnecessary surgical risks,** longer recovery times, and higher out-of-pocket expenditures.
- **Ignoring the Social Determinants of Health:** Clinical quality cannot completely offset the deep socio-economic inequalities that women face before they even become pregnant.
  - According to NFHS-5, a staggering **57% of women (aged 15-49) in India** are anemic. Driven by **patriarchal household dynamics** where women often **"eat last and least," chronic malnourishment** means minor obstetric bleeding can easily turn fatal.
  - Despite legal frameworks, **23.3%** of women aged 20-24 years were married **before the age of 18 (NFHS-5).** Teenage bodies are **biologically less prepared for childbirth,** leading to a higher incidence of cephalopelvic disproportion and prolonged labor.

What Measures can Strengthen Maternal Healthcare?

- **Adopting Respectful Maternity Care (RMC):** Moving beyond clinical outcomes to prioritize the dignity of the mother. This includes training healthcare staff in "soft skills" to eliminate obstetric violence, ensuring privacy during labor, and allowing a "birth companion" of the woman's choice, which is proven to reduce stress and complications.
- **Midwifery-Led Care Units (MLCUs):** Establishing a dedicated cadre of **Nurse Practitioners in Midwifery (NPM)** . Midwives can handle low-risk natural births, which decongests tertiary hospitals and reduces the "epidemic" of unnecessary C-sections.
- **Blood Storage and Transport:** Expanding the **"Hub and Spoke" model** for blood banks.
- Every high-delivery-load facility must have a functional blood storage unit to treat Postpartum Hemorrhage (PPH) within the "golden hour."
- **Digital Tracking:** Leveraging the **Pradhan Mantri Surakshit Matritva Abhiyan portals** to identify "High-Risk Pregnancies" (e.g., severe anemia or hypertension) early and ensuring they are referred to specialists long before labor begins.
- **Combatting Maternal Anemia:** Strengthening the **Anemia Mukt Bharat** strategy through mandatory **iron-folic acid (IFA)** supplementation and, more importantly, community-level nutritional counseling to change the "women eat last" household dynamic.
- **Empowering ASHA and Anganwadi Workers:** Providing these frontline workers with better diagnostic tools (like digital hemoglobinometers) and higher incentives for tracking postpartum care, not just the delivery itself.
- Conclusion
- India has made strong progress in maternal healthcare, but the final stretch remains challenging.

Achieving SDG 3.1 will require shifting focus from infrastructure to quality, equitable care, along with improving women's social and nutritional status.

Drishti Mains Question:

Q. "India has improved maternal health outcomes, yet structural and social challenges persist." Critically examine

Frequently Asked Questions (FAQs)

1. What is Maternal Mortality?

Death of a woman during pregnancy or within 42 days of termination due to pregnancy-related causes; a key public health indicator.

2. What is Maternal Mortality Ratio (MMR)?

Maternal Mortality Ratio (MMR) is the number of maternal deaths per 100,000 live births; it reflects the quality of maternal healthcare.

3. What is the SDG target for maternal health?

Sustainable Development Goal (SDG) 3.1 aims to reduce global MMR to below 70 per 100,000 live births by 2030.

4. What are the major causes of maternal deaths in India?

Haemorrhage, infections, and hypertensive disorders are the leading preventable causes.

5. What is Respectful Maternity Care (RMC)?

Respectful Maternity Care (RMC) ensures dignity, privacy, informed consent, and emotional support for women during childbirth.

[Watch Video on YouTube: ▶ **https://www.youtube.com/embed/EmlIVFarLUA**]

UPSC Civil Services Examination, Previous Year Question (PYQ)

Mains

Q. In order to enhance the prospects of social development, sound and adequate health care policies are needed particularly in the fields of geriatric and maternal health care. Discuss. (2020)

Birth Anniversary of Dr. Bhimrao Ramji Ambedkar

★ ★ ★ ☆ ☆

Source: PIB

Why in News?

On 14 th April 2026, the nation celebrates the **135 th Birth Anniversary** of **Dr. Bhimrao Ramji Ambedkar ,** the chief architect of the **Indian Constitution,** a pioneering social reformer, an eminent jurist, and a towering intellectual who dedicated his life to the eradication of social inequality in India.

Who was Dr. B.R. Ambedkar?

- **Early Life:** Born on 14 th April 1891, in Mhow, Central Provinces (now Madhya Pradesh), into the **Mahar caste** . He faced severe socio-economic discrimination from a young age.
- **Education:** He was a brilliant scholar, earning doctorates in economics from both Columbia University (USA) and the London School of Economics (UK).
- **Political Career:** He served as the **First Law Minister of Independent India** and was the Chairman of the **Drafting Committee of the Constituent Assembly** .
- **Organizations and Literary Works: Dr. B. R. Ambedkar founded key organizations such as the Bahishkrit Hitakarini Sabha (1924) for socio-economic upliftment of depressed classes, the Independent Labour Party (1936) for workers' rights, and the Scheduled Castes Federation (1942) to advocate for Dalit rights.**
  - He also used print media through journals like *Mooknayak* (1920), *Bahishkrit Bharat* (1927), *Samatha* (1929), and *Janata* (1930) to spread social awareness.
  - His major works, including *Annihilation of Caste* (1936), *The Untouchables* (1948), *Buddha or Karl Marx* (1956), and *The Buddha and His Dhamma* (1957), remain foundational texts on social justice and equality.
- **Posthumous Honor:** He was awarded India's highest civilian honor, the **Bharat Ratna** , in 1990.

Do You Know?

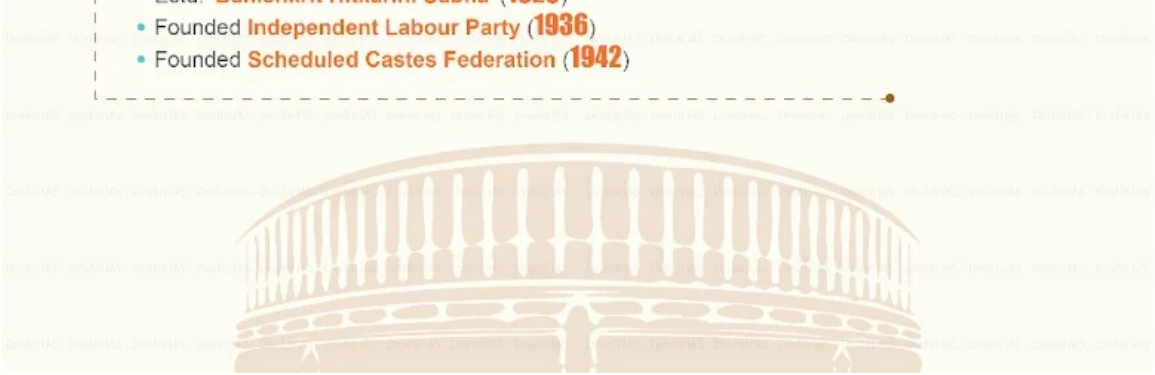
- The Government of India has developed **five key sites** associated with B. R. Ambedkar as the **'Panchteerth'** to preserve and promote his legacy.
  - **Janma Bhoomi:** Mhow, Madhya Pradesh (Birthplace).
  - **Shiksha Bhoomi:** London, UK (Where he stayed while studying).
  - **Deeksha Bhoomi:** Nagpur, Maharashtra (Where he embraced **Buddhism** in 1956).
  - **Mahaparinirvan Bhoomi:** Delhi (Where he breathed his last on 6 th December 1956).
  - **Chaitya Bhoomi:** Mumbai, Maharashtra (Place of cremation).

What are the Key Contributions of Dr. B.R. Ambedkar?

- **Social Reform & Crusade Against Caste:**
  - **Mahad Satyagraha (1927):** He led the **Mahad Satyagraha** to assert the rights of the untouchables to **draw water from the public Chavdar tank in Mahad, Maharashtra.**
  - **Temple Entry Movements:** He actively led movements advocating for the **right of marginalized castes to enter Hindu temples** (e.g., Kalaram Temple entry movement, 1930).
  - **Poona Pact (1932):** Following **Mahatma Gandhi' s fast unto death against the Communal Award** (which granted separate electorates for depressed classes), Ambedkar signed the **Poona Pact, securing reserved seats for the depressed classes within the general Hindu electorate.**
- **Constitutional & Democratic Contributions:**
  - **Drafting the Constitution:** As Chairman of the Drafting Committee, he ensured that the Constitution incorporated robust safeguards for minorities, marginalized sections, and women.
    - Ambedkar believed India's foundation should rest on **liberty, equality, and fraternity** , inspired by the French Revolution (1789-1799).
  - **Article 32:** He famously called **Article 32 (Right to Constitutional Remedies)** the **"heart and soul"** of the Indian Constitution.
- **Economic and Labor Reforms:**
  - **Foundation of RBI:** The conceptual framework for the Reserve Bank of India (RBI) was based on the guidelines presented by him to the **Hilton Young Commission** , derived from his book ***"The Problem of the Rupee: Its Origin and Its Solution."***
  - **Labor Rights:** As the Labour Member of the Viceroy's Executive Council, he introduced significant reforms including the **reduction of factory working hours (from 14 to 8 hours)** , equal pay for equal work irrespective of gender, and maternity benefits.
- **Women's Empowerment:** He introduced the **Hindu Code Bill** in the Parliament to grant women equal rights in inheritance, marriage, and divorce.
  - When the bill was stalled due to orthodox opposition, he resigned from his post as the Law Minister in 1951.
- **Religion:** Navayana (New Vehicle) Buddhism, founded by B.R. Ambedkar in 1956, is a reinterpretation of Buddhism that emphasizes social equality and class struggle over traditional spiritual doctrines.
  - It rejects core Buddhist doctrines like **the Four Noble Truths, karma, rebirth, nirvana, and**

monasticism , considering them pessimistic and irrelevant to social justice.

- **Dhammachakra Pravartan Din** is observed annually on 14 th October to commemorate the historic mass conversion of B. R. Ambedkar and his followers to Buddhism at Deekshabhoomi, Nagpur, in 1956.



Click here to Read: **Comparison of Gandhi and Ambedkar's Philosophies**

[Watch Video on YouTube: ▶ **https://www.youtube.com/embed/VyDQSwHfoHs** **]**

UPSC Civil Services Examination Previous Year Questions (PYQ)

Prelims

Q. Which of the following parties were established by Dr. B. R. Ambedkar? (2012)

1. The Peasants and Workers Party of India
2. All India Scheduled Castes Federation
3. The Independent Labour Party

Select the correct answer using the codes given below:

(a) 1 and 2 only
(b) 2 and 3 only
(c) 1 and 3 only
(d) 1, 2 and 3

Ans: (b)

Mains

Q. Mahatma Gandhi and Dr. B.R. Ambedkar, despite having divergent approaches and strategies, had a common goal of amelioration of the downtrodden. Elucidate. **(2015)**

9th Indian Ocean Conference

★ ★ ☆ ☆ ☆

Source:TH

At the **9 th Indian Ocean Conference** in Mauritius, India reaffirmed its firm opposition to the targeting of **civilians, infrastructure, and commercial shipping** amid the escalating **2026 West Asia conflict , while calling for de-escalation and stability.**

- No delegate attended from the Maldives, as diplomatic ties between **Mauritius and the Maldives** have been suspended over territorial differences related to the **Chagos Islands.** Meanwhile, Bangladesh emphasized the need to **“revitalise SAARC ” to strengthen regional cooperation.**
- **Impact on the Indian Ocean Region (IOR):** India highlighted the severe economic implications of the West Asia conflict on **energy security, trade, and fertilizer supply chains** across the IOR, stressing the absolute necessity of maintaining **safe and unimpeded maritime navigation** .
- **India-Mauritius Energy Pact:** To safeguard against regional volatility, **India is finalizing an agreement to supply oil and gas to Mauritius** , strategically bolstering the island nation's energy security.
- **Indian Ocean Conference: It was launched in 2016 by the India Foundation in collaboration with regional think tanks and has grown into a key consultative platform with participation from over 40 countries.**
  - It serves as a flagship forum for discussing regional affairs in the IOR and promotes cooperation under the vision of Security and Growth for All in the Region (SAGAR) .
- **Indian Ocean Region (IOR):** It is a vast maritime region spanning over **70 million sq. km** , stretching from Africa to Australia and covering nearly **20% of the world’s water surface** .
  - It is a vital global trade hub where **half of the world’s container ships, one-third of bulk cargo, and two-thirds of oil shipments** pass through.
  - The region connects **35+ littoral states** and is home to nearly **40% of the global population** , highlighting its immense strategic and economic significance.
  - India plays a critical role in the IOR as a **net security provider and primary stabilizing force, driven by the SAGAR (Security and Growth for All in the Region) vision.**

Read more: India's Renewed Focus on Indian Ocean Region

Deposit Tokens and Asset Tokenisation

★ ★ ☆ ☆ ☆

Source: BL

As global finance rapidly shifts toward **programmable, always-on settlement systems,** discussions have emerged on how India can safely modernize its **banking sector,** highlighting **deposit tokens and real-world asset tokenisation** as the next evolutionary steps beyond its successful **digital public infrastructure (like UPI).**

- **Deposit Tokens:** These are digital representations of **bank deposits** issued on permissioned **blockchain networks ,** fully backed by traditional deposits and subject to regulation, enabling **real-time settlement, programmability, and no additional credit risk** .
- **Advantages over Unregulated Crypto:** Unlike private **cryptocurrencies** , deposit tokens are direct claims on a bank's balance sheet.
  - They extend the regulated banking framework into a programmable digital layer, enabling **near-instant settlement, atomic delivery versus payment (DvP), and automated reconciliation** without introducing new credit or liquidity risks.
- **Transforming Banking Operations:** For Indian banks, the adoption of deposit tokens means **interbank settlements, treasury operations, and large-value corporate payments** can transition from delayed batch-based processes to real-time, instantaneous rails.
  - It also promises to make **cross-border transactions** faster, cheaper, and more transparent.
- **Real-World Asset Tokenisation:** It involves converting real-world assets like **real estate, gold, and infrastructure** into digital tokens.
  - It addresses India's liquidity constraints by allowing traditionally illiquid assets to be **fractionalised, transferred, and used efficiently as collateral** .
- **Synergy for Market Efficiency:** When **tokenised assets are paired with bank-issued deposit tokens, t** rades can settle instantly in regulated digital money.
  - This drastically **reduces counterparty risk** , lowers operational costs, and broadens market participation under clear regulatory guardrails.
- **Regulatory Roadblocks:** Currently, many tokenisation initiatives are confined to regulatory sandboxes.
  - To scale this, India urgently needs **regulatory clarity around foreign exchange (Forex) laws, Anti-Money Laundering (AML), KYC norms, and cross-jurisdictional compliance** for blockchain-based instruments.
- **Strategic Imperative for India:** By proactively establishing frameworks for regulated digital money, especially for **trade finance and institutional settlements,** India has the strategic opportunity to **shape global standards** .
  - Delaying this transition risks capital and innovation migrating to other, more technologically integrated jurisdictions.

Read more: Central Bank Digital Currency